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Title of the article: *Between Negative Stigma (Cultural Deprivation) and Positive Stigma (Learning Disability): The Historical Development of Two Special Education Tracks*

The content extracted is as follows:

Introduction

Thus, the LDs discourse (much like the cultural deprivation discourse) could have been used potentially from its very inception as a divergent special education track serving a distinct social population.

Revealing the conceptual, historical, and cultural foundations of the current Israeli, LD discourse can eventually contribute to the development of a more theoretically complex discourse by exposing its cultural and political context as well as the social forces shaping it in the present and possibly in the future.

On the Social Construction of LDs and Cultural Deprivation

Another concept that was very popular during those years was cultural deprivation, a designation given mostly to children who belonged to families of Middle-Eastern origin of limited means.⁹ Contrary to the other concepts which were conceived as emanating from a biological etiological infrastructure, the concept of cultural deprivation was believed to originate from an environmental etiological infrastructure (even though it still also had biological manifestations as a consequence of the environment characteristics).^{10,11,12} A review of the books and articles in Hebrew on the subject revealed that, since the adoption of the term LD, the term LDs has become popular and has replaced the various aforementioned concepts (concepts which came to be regarded anachronistically as its predecessors).

Kirk de ned LDs thus: pedagogical discourses (including the parallel cultural deprivation discourse) were turned upside down: Academic dif culties that were previously identi ed with mental retardation, with intellectual mediocrity, with laziness, or with moral weakness, have begun to be perceived as indicative of exactly the opposite traits!

Moreover, the neurological susceptibility discussed was conceptualized in an a-deterministic way, a conceptualization that facilitated the reception of the new discourse among groups of worried parents.

Juxtaposing the Two Syndromes

the disability discourse) was attributed categorically to culturally deprived children.²⁰ It must be remembered that in the formative period of this discourse on LDs, the dominant opinion in this eld determined that the children with organic issues have standard intelligence and standard motivation.

Etiological Differences

In stark opposition to this, in the LD discourse, there is a complete disregard of the cultural and socio-economic contexts as a relevant component in the conceptualization or the construction of the disabilities.

So, under the aegis of the burgeoning discourse on cultural deprivation, its expert proponents developed a kind of index of negative characteristics which related to the various aspects of the culturally deprived student s personality.

The defects which characterized these qualities were together identi ed as symptoms of a defective self as noted by Frankenstein (1969).²⁶ In this way, the culturally deprived discourse produced a syndrome whose environmental (cultural and socio-economic) roots are expressed in the personality structure of those af icted with it.²⁷ Summarizing this part, the current article seeks to point out the fact that in parallel to the drafting of the aforementioned cultural deprivation pro le, there was a simultaneous drafting of a clinical pro le that eventually came to be known as learning disabled. The representations of the self of both of these special education subjects were produced as distinct and different.

The Importance of Sensitivity to Parents

A repeated motif in the LDs discourse deals with the great sensitivity that must be displayed in dealings with the parents of children with dif culties.

The explicit statements about the importance of sensitivity were supported by a series of textual mechanisms whose intertwining created a conventional rhetoric of empathization with parents, their children, and the suffering of both.

Rhetorical mechanisms of empathization were directed at minimizing negative connotations, which were usually identi ed with disabilities in the structure and function of the brain.²⁹ Another central element which gave expression to the sensitivity and the identification of the experts with the parents of children with dif culties can be noted in the demoralization of learning problems and dif culties in the discourse on LDs.

The development of the disability discourse was characterized by the disappearance of moral indicators for disabilities; a disappearance that may have eased the espousal of the discourse for many groups of parents.³¹

The Image of the Good Parent: The Adequate and Helper Parent

In this way, the experts encouraged the inclusion and integration of parents into the disability discourse while insuring themselves a position of superiority in it by creating an obvious dependency on them on the part of the parents who desire to update themselves frequently with the burgeoning knowledge.

Parent Expert Cooperation

The authors' recommendations did not only relate to the ways in which the parents ought to spend their free time with their children, whether it concerned their methods of entertainment or their teaching methods or their emotional and social formation within the home.

Furthermore, many experts explicitly position the parents as their partners in real understanding of the children's difficulties, and make themselves the allies of the parents in their efforts to cope in the face of the refusal of the teachers and the education system in general to acknowledge that LDs are the explanation for the difficulties.

The Good Parent as a Source of Finance

The expert discourse assigns the good parent an additional important function: ongoing and long-term financing of the disabled child's treatment.

This function has been repeated over and over from the inception of the field of disabilities in Israel and is expressed in writing thus: Parents of learning disabled children are supposed to get in touch with remedial teachers, teacher of extra lessons, counselors, doctors or nurses, and paramedical therapists (speech therapists, occupational therapists, etc.) (Segal 1969).

For this reason, the expert discourse outlined the figure of the ideal parent: an involved parent who devotes time and energy to the educational act, while at the same time being aware of the importance of demarcating this involvement and delegating the management and organization of the child with difficulties to the expert.

Guilt and Responsibility

Despite the doxic biological conceptualization of LDs (as nervous disorders which have nothing to do with defective education or inappropriate environmental conditions), the written works of the experts have created the moralistic imperative (usually delicately formulated) whereby parents must take care of the proper conduct of their children's academic career as well as the appropriate management of their cognition, emotions, and behavior.

The parents' lack of obedience, even implied, is interpreted as a significant factor in worsening the disabilities and in causing the potential defects to become activated. The discourse on disabilities shapes and reflects the various models of responsibility, including parental responsibility and the division of labor between various social agents: private experts, state educational institutions, parents, and the disabled. The textual analysis reveals the fact that the parents, as those responsible for the failure of their children, are placed in positions that are often susceptible to supervision, evaluation, and criticism on the part of the experts.

The ongoing sophistication of the experts in terms of therapeutic and pedagogical models creates an abundance of age-specific prescriptions from which the parents must learn rules of behavior and conversation with their children.

Problematic Parents of the Culturally Deprived

This profile places the latter in a negative light, to say the least the characteristics associated with him in the literature include cognitive characteristics (obstinacy, lack of logic, lack of education, ignorance, and low achievement); personal characteristics (lack of belief in his own strength, passivity, low frustration threshold, and insecurity); family characteristics (lack of understanding of family needs, lack of emotional relationship with the children and spouse, authoritarianism on the part of the father, and submissiveness on the part of the mother); and other characteristics (cultural backwardness, reduced connection with the surrounding culture, and dependence on external authority).

An example is Eiger's remark that the impersonal attitude to the child does not mean that the parents do not express love, affection, or concern for their children, but because of the weak self of the parents, the expression of emotion is impulsive, uncontrolled, and lacking in constancy which is the reason for the rapid transitions from love to rage, from neglect to devotion, etc. This attitude does not allow the child to feel the constancy of the parents' attitude and so he is not able to internalize it (Eiger 1975).

To a large extent, the Mizrahi parents were identified as being to blame for their children's backwardness.

It is paradoxical that these parents were found to be responsible for their children's failure because the cultural deprivation discourse itself emphasized the class and cultural marginalization of these parents in the social structure and blurred the socio-critical potential by its very presence and emphasis on this structural marginalization.

In well-known senses, the discourse on cultural deprivation is antithetical to that of brain

damage and, later, to that of LDs. The latter proposed a different special education taxonomy.

As mentioned, they also emphasized the importance of expert sensitivity for the parents' distress and asserted that there was no parental blame or responsibility in the engendering of disabilities.³⁷ The two discourses exemplify two distinct types of parental cultures of blame and responsibility: The LDs discourse reflects an inclusive and integrative model of blame and responsibility.

The parents are not causally blamed for engendering their child's disability, but still they are responsible for the management of that disability, whereas the cultural deprivation discourse reflects an exclusive model of blame and responsibility: the parents are causally blamed for engendering their child's disability, while not being responsible for the management of that disability.

Both of the discourses assign a central but different role to the parents due to the parents' important position as mediators between the public spheres of the medical and educational discourse and the private sphere of the family.³⁸ Confining their expertise to those socially distinguishable parents whose so-called individual qualities were valid for them.

Based on these qualities, in effect, established, educated parents are identified in the expert literature, as potential, natural partners, not only in individual therapy for the disabled children but also in studying and dissemination of the body of knowledge about LD itself. Not every parent was able to be a partner for the experts in therapy, consultation, learning, and dissemination of the discourse.

This appearance of discursive, conceptual distinctions, and professional and institutional division of labor valid for social, ethnic, and class distinctions serves as the historical case study of the discourse on cultural deprivation in Israel.

Many have written about the formation of the segregationist educational ideology during the 1960s focusing mainly on the now discredited discourse of cultural deprivation.

That ideology developed against the background of the incoming waves of immigration to Israel from states which were conceived as backward, the negative perceptions of many of the immigrants on the part of the indigenous population, and the widespread local public belief in the validity of the I.Q. test.³⁹ The simultaneous existence of the LDs discourse (a discourse that emerged in Israel in those very years) highlights an otherwise hidden aspect in the historical formation of the aforementioned segregationist ideology: The currently rejected widely critiqued cultural deprivation category did not emerge in an educational vacuum devoid of alternative patho-educational conceptualizations.

The critical literature is full of descriptions of the low expectations of the educational experts of the culturally deprived children.

And, in the same vein, we can broaden the question and consider whether there were any differences between the expectations of experts of culturally deprived children and their expectations of learning disabled children?

In retrospect, it is surprising to realize what a large difference there was between the expectations of the disabilities experts regarding the children who had difficulties assessed as being based on an organic background and their expectations regarding the culturally deprived children.

However, while concerning the culturally deprived children, the disabilities experts adopted the customary and accepted motifs: failure expected in advance and the need to lower expectations, concerning the learning disabled children they adopted different motifs.